Monday

Thera orals 2025

Good luck everyone!!!

Are we allowed pen and paper? YES YOU ARE!, didn't get paper for mine so i think it is supervisor dependent
How many cases do they ask? Tbh i got asked 4 but we had time, so i think case report + 2 cases (anterior + posterior)

- Some sups ask case reports
- Others don't

MONDAY MORNING

SUPS: Daisy and Angelica
CONDITIONS: AAC, marginal keratitis, SAC

HEYYY GUYS!!!!!! I'll try to write down everything I remember…

- **Case 1:** Sat down and started asked about my case report. I had NTG and got over 75 so don't count on the fact that you'll only get asked if you got less than 75. Questions were mainly related to the comments they make on your case report so brush up on that. I got probed on CCT + IOP so understand that relationship well.
- **Case 2:** Showed me a picture of both eyes which looked asymmetrical. OD had an opaque cornea and edematous lids, OS looked clear. Told me the pressures were more elevated in the right eye and asked me for differentials. Tbh i was so focused on the oedema that my mind just made me think i was dealing with a preseptal cellulitis or orbital cellulitis situation. I got probed to focus on the cornea. Then she asked me what do i expect to see in an AAC a ttack. (lowkey i cant remember what came first but somehow got probed towards AAC)...discussed management the whole call for acetazolamide, iop lowering drops and prednisolone acetate situation..then we moved on. Everything happened so fast, i feel they just wanted to show me everything and we went into nothing in depth so had no chance to discuss CI or SE or anything like that.
- **Case 3:** showed me a pic of one eye that had an inferior stromal infiltrate. I asked about symptoms and she said like mild pain (so vague) and not a contact lens wearer, i asked if it stained and she said no, then asked for my differentials. I lowkey could only think about marginal keratitis so i told her idk others but mentioned that i'd be sus of MK. asked how id manage and i spoke about lid hygiene + steroids…forgot prophylactic AB, asked review period (i said next day because surely seeing them sooner if better than later)
- ***there was 2 mins left, they looked at each other and questioned if they should ask another case and low and behold they did sooooo - Case 4:** showed me a inferior palpebral conj which looked red and papillae and px symptoms were itchiness and irritation. I asked if they have allergies and she said yeah to hayfever. Then i kinda said PAC then changed it to SAC. she asked me about management so

i said combo mast cell stabiliser and antihistamine olopatadine (patanol) 0.1% two times a day. She asked review period.

TO SUMMARISE: no CI or SE were asked. It was pretty much, tell me what you see, ddx, diagnosis, management and review. You've got this guys!!!!!!!!!!!!!!!

What's CI and SE? Contraindications and side effects sorrryy lol

FUNGAL, AKC/SAC, case report

Fungal is a gomo! Nantamycin 5% filamentous, amphetericin B for candida

Monday 9:20am

1) case report on POAG
2) PDS
3) Fungal keratitis
4) AKC

Monday am (Both case reports, AKC/SAC, Fungal Keratitis)

1) Sat down and Angelica asks "what did u learn from ur case report?"
2) I knew this was a chance so i started yapping about PG non stop
3) Angelica asked me some questions like what are the signs of burn out phase? (reduced pig, pigment reversal sign, stabilise IOP)
4) What would be some difference in symptoms in burn out stage?
    a) Exercise - No pigment showering effect so no HAs
5) What surgerys for PG
    a) SLT very effective cus it targets the pigment cells
    b) What else? What about the reverse pupillary block?
        i) LPI can release the pressure but some studies have said its ineffective
        ii) Angelica was happy
6) Why treat both eyes if one eye is PG and one is just PDS
    a) As prophylactic blajblblah
7) SUDDNELY she switched the topic and said In the context of unilateral glaucoma what would change in your work up?
    a) This thew me off D: I didn't know this but i think she wanted me to say check for neurological causes and refer for MRI??? Cus the next question she asked me is what would prompt an MRI
        i) I talked about neurological signs but idk this part was so confusing
8) **Talk about the management of a POAG**
    a) I talked about pga (first line) - side effects, contraindications, punctual occlusion, refer to opthal in 4M!! <- important
9) **Case report 2:**
    a) Moved on to my episcleritis case report
    b) What would prompt a systemic work up?
        i) High recurrency
        ii) Nodular etc
        iii) Full blood work up
    c) What are management options for episcelritis other than steroids

i) Oral nsaids

d) Why would you choose one over the other?
   i) Steroids - rapid, direct effect on the eye
   ii) Oral NSAIDS - systemic, might taker longer for effect, could be useful if contraindicated for steroids - WHAT ELSE? When you want to treat more inflammation..? WHEN WOULD U WANT TO TREAT MORE INFLAMMATION? (or smthing like that) - When it is long standing and recurrent? Nodular type ?? and they nodded and moved on

10) Daisy showed me a pic of big central ulcer + infiltrate with feathery margins and said "50 yr old male, did gardening and got this, What is ur differential"
   a) Fungal, MK, acanthamoeba

11) How does acanthemoeba keratitis look like
   a) Talked about pseudodendrities, perinueral infiltrates …

12) Waht ur diagnosis
   a) Since highly likely vegetative matter trauma + feathery margins → fungal kerattis

13) OK what ur managmeent
   a) MUTT trial - refer for natamycin 5% for filamentous, better than voriconazole 1%
   b) For Candida.yeast - amphtoericin B idk concentration so i didnt say

14) Whats the dosage
   a) Hourly until reepi-thelisation and then qid after that

15) At night too??
   a) No only when awake

16) Next case: shows me a hella blurry pic of inferior palpebral conj with like a few papillae
   a) Guy with hayfever comes in with itchy eyes what is this
   b) I said i can see papillae
      i) We went in to papillae vs follicles (what cuases papillae, what causes follices, how do they look different)

   c) So whats ur final diagnosis
      i) Do they have any atopy???
         (1) Daisy said they could?? But she didnt give me a definite answer i think the main thing is treating the allergy anyway so diagnosis might not be that important (hopefully..)
      ii) I said my two main ones would be akc and sac but akc would be more chronic but tx would be the same regardless
         (1) I chose SAC at the end cus hayfever.. Idk maybe i shouldve said AKC ? but not much info for me to diagnose AKC
   d) Management
      i) Avoid allergen, palliative, combo drops
   e) What is the max duration u can use combo
      i) 14 weeks
   f) Whats ur review period
      i) I said in a week cus it takes a week or two to see acutal changes in signs

ii) But then i got scared and said but i will call them the next day to do a telehealth and see if the symptoms have improved

Main take away

- Study ur case reports (BOTH OF THEM WERE ASKED although i got 90 for PDS)
    - They ask u questions from themselves or questions in the actual feedback so study that
- Prepare for blurry photos…. Like super blurry

MONDAY AFTERNOON

SUPS: Isabelle Jalbert and Elizabeth Wong (omg) so lucky

1:20pm:

- **Case 1: CL-related bacterial keratitis**
    - A photo with a large corneal infiltrate (~2mm) very close to the visual axis.
    - Px is a CL wearer, symptomatic since yesterday
    - DDx: basically all keratitis - bacterial, fungal, acanthamoeba, sterile infiltrate etc
    - How to differentiate/Workup:
        - Mentioning the size (large size is more likely infectious than sterile), the shape of the size (fungal is more fluffy margin, acanthamoeba is ring infiltrate)
        - Also I mentioned confocal microscopy - Don't recommend saying this if you are unsure the exact mechanism of confocal microscopy because I got interrogated T.T
    - Then, Isabelle said "Okay if we decide that it is microbial, how do u manage"
        - I went with ciprofloxacin 0.3% monotherapy (I think it was correct since she didn't question it, but now in retrospect, I think if I mentioned fortified dual, it is also acceptable as well)
        - Loading dose q15min for 6 hours, than q30min for the rest of the day. Then q1h for 2 days, etc
        - I mentioned briefly that px needs to dose at night as well but she didn't go any further regarding compliance.
    - Follow-up:
        - Next day to confirm no worsening, check with NaFl to see the re-epithelisation
    - Additional therapeutic considerations:
        - I said steroids due to the risk of scarring and infiltrate is so close to visual axis
        - She asked which one. I said Prednerferin Forte.
        - She asked why PredForte. I said because it has the best corneal penentration

        - She kinda probed it by asking “What is the composition of Prednerferin Forte?” So basically, prednisolone 1% + 0.12% phenyl. Then she probed more “which type of prednisolone?”. She wanted to hear that in Prednerferin Forte, it is prednisolone acetate, which is lipophilic hence better corneal penetration.
    - She asked why I chose cipro.
        - I said since it is a broad spectrum AB with good gram-ve coverage. Considering that the px is a CL wearer, they are at higher risk of infection by Pseudomona.
        - Then she asked why cipro over ofloxacin. I actually went blank and could not answer. But the correct answer is that ofloxacin has a higher risk of corneal perforation
    - I didn’t get asked but for bonus points (and if you get asked) maybe mention ceasing CL wear and rethinking CL modality. But Isabelle let me slide.
- **Case 2: Ocular Hypertension**
    - A very pixelated, blurry screenshot of an Optos T.T
    - Essentially everything was unremarkable
    - IOP was 29 (Elizabeth told me to assume that everything is bilateral)
    - Workup:
        - CCT - normal
        - Gonioscopy - normal
        - Anterior slit lamp for any secondary causes - unremarkable
        - OCT - Lizzie said it was unremarkable (wasn’t given any OCT scans)
        - VF 24-2
            - I got a VF result but it was all clear.
    - Tentative diagnosis: Ocular hypertension
    - She asked me if I will treat, what risk factors and based on what major study.
        - OHTS study
        - I said thinner CCT is a risk factor but since my px CCT is normal
        - Raised IOP is a risk factor since 1mmHg reduction is less 10% risk of progression
            - And she was like “are you sure?” and I said I am sure 😀
        - Age can be a factor
        - Most importantly, first-order FHx of glaucoma
    - Then treatment: if px comes back and IOP is still raised and reaches the 30s range, then treated because according to OHTS, treatment reduces the risk by 50%
        - PGA. Side effects: increased iris pigmentation, eyelash growth, periorbital fat loss
        - If the patient doesn't want to take drops, SLT because LiGHT trial says that SLT is a better IOP control, in 74% of patients.
    - But the cherry on top that threw me off literally and I fumbled so bad 😭 “Except for glaucoma, how does raised IOP affect the eye?”
        - Risk of angle closure, halo, corneal oedema, optic neuropathy - ngl I fumbled quite hard, just a bunch of gibberish so I am terrified
- Overall gist I got from my oral:
    - Be confident people! They might make questionable facial expressions and “Are you sure?” - But if you are sure, just stand on business!

- They will kinda lead you to the information they wanna hear so if they didn't ask for something, don't panic and just let it go. 20 minutes is not enough for lots of questioning and probing anyways!
- But GOOD LUCK PEEPS! U will be fine. Just breathe and think twice. U can fix a previous answer that u think it was wrong.

CASE: NTG + AAU (w/ hypopyon)
Image (colour & FAF of posterior pole) described what I saw
What tests would you do (asked about water drinking test and what you'd expect to be abnormal)
Name the papers associated with it and their findings
Referral guidelines (asked if we need to follow RANZCO)
Mechanism of NTG
Treatment
Why wouldn't SLT work well
If PGA didn't work what would you do
CI of BB
what target would you do if normal pressure was 14 mmHg and water drinking showed 18mmHg

- Do we do 30% reduction from 18?? Help pls
- Target IOP ≈ 25–30% below baseline 25–30% reduction from 14 mmHg → 9–10 mmHg,

What if your patient didn't want drops?

Shows picture of AAU w/hypopyon
What do you see? I asked history questions first
Went through blurry photo
Causes of AAU and percentages
What would you do?
Refer bc hypopyon (asked how quick i would refer)
Went through review of systems
treatment (asked me what was on and what was off PBS)
Prognosis? How long do you expect it to last?

i didn't talk get to talk about Ddx, taper schedule, or checking IOP though. But they said they're fine with leaving it there at the end? we still had 5mins

it was more self guided and they would ask questions on what you mention

Monday 2:40
Case 1: VKC w shield ulcer
Isabelle described to me a 12yo boy with intense itching eyes and giant papillae

1. ddx?

I just said SAC PAC and VKC they seemed happy enough

2. What tests do u wanna do now?

Show me the cornea

3. *GIANT GRADE 2 CENTRAL SHIELD ULCER* what is ur tx

Prednisolone acetate 1% pulse therapy q1hr for 4 days, then qid 4 days
Debridement and AMT (I didnt actually say debridement but i should have AAAA)

4. Contraindications of corticosteroids?

(I totally blanked here) but said HSV and active infection (she was not satisfied with this) what else could be said?

5. Mechanism of action of corticosteroids

Inhibit prostaglandin synthesis and keratocyte + fibroblast activity (block inflammation pathway yap yap yap BIG FUMBLE I COULD TELL SHE WAS DIGGING FOR SOMETHING EEK

6. (isabelle again) WHAT ARE THE CONTRAINDICATIONS OF STEROIDS?????

This is where i said about how kids react more to steroids and more prone to iop rise and cataracts.

7. ???WHAT ELSE WOULD YOU DO IN THIS CASE???? (she was digging hard i was not going well obviously)

I went on a ramble about olopatadine and then she stopped me to talk about what else i would do for the ULCER

8. Do you expect there to be an epithelial defect?

…Yes

9. …So what do u do?

OHHHHHHH Prophylactic antibiotic CHLORSIG QID UNTIL EPITHELIUM HEALS AND 3 DAYS AFTER

10. How long do you expect this to take this to heal?

(I had no idea) I said 1-2 weeks and she gave mega side eye

11. How quick would an RCE heal…?

Within 5 days. Rapidly

12. So will this also heal rapidly?

No because shield ulcers are huge physical barriers with slower epithelial healing (???? she gave me huge side eye HELP)

Case 2: PDS/PDG

30y/o male with pressures of 26 both eyes. Routine eye exam. Then shoves me a blurry ahh photo of a heavily pigmented TM

1. Whats your diagnosis (not even asking for ddx HAHAH) and what else do you expect to see

Krukenbergs, mid peripheral TID, posterior bowing of iris etc.

2. What is a more RARE finding of PDS?

I was like girl if its rare I dont know it HAHAHHAHAHA SORRY

3. What do u wanna do now

Show me back of the eye pls!

4. *Describes to me* LE only superotemporal RNFL thinning and NRR loss. What VF defect do you expect to see?

Inferior nasal step

5. *Shows me a textbook inferior nasal step* Grade this VF

It was grade 1 (early) according to mills criteria

6. Are you treating this patient?

Yeah duh. Did the whole PGA yap and EMGT percentage reduction

7. Are you treating the RE too? It has no VF loss but still pressures of 26 (OHT)

I then quoted OHTS and said bc we are doing drops in the LE its kinda just convenient to put the drops in both plus PGA cosmetic side effects better to be symmetrical

8. Ur patient doesnt want cosmetic side effects. What now?

Well i said BB no no bc hes young and im assuming he exercises (bradycardia and bronchospasm) so then I brought up SLT

9. Does SLT work in PDG?

I said yes but with lower laser power (I did not yap about efficacy or how tx may not last as long AAAA WHOOPS)

10. How does SLT work?

(I actually dont know) but i said goes pew pew and targets pigment and TM debris lollllllll and then time was up!

Good luck everyone xoxoxo

3:00 MK + POAG

NO CASE REPORT QUESTIONS

EW - Case 1: POAG

- PX comes in with slightly high IOP: R22 L24
- Shows me pictures of RNFL + GCL scan
- R: OHT with no defect, L: clear wedge defect superior and inferior - matching with GCIPL
- Elizabeth asks me to describe
- Then, shows me a Left VF with nasal step and central scotoma
- Describe VF - what other test would you do when there is a central scotoma?
- 10-2 Required to check
- Grade the VF - MD -6.00 ish, MILLS - early~moderate, however centre involving, so double checking repeatability is crucial. Think its good to explain your rationale (your mx does not change tho)
- Management? PGA - given that Px is not preggo, no active inflammation. And Refer to Ophthal within 4M of prescribing med
- Would you do both eyes? You could, PGA cosmetic reasonings and OHT. I talked about OHT study and low progression. Given that there is no defect at all (both structure and functional) in right eye, I would focus on LE more than RE at this stage.
- Mechanism of POAG? - talked about OPP, lamina cribrosa damage, ischemia, mechanical damage from cupping, axoplasmic disturbance, extracellular matrix instability - ultimately causing ‘slow death of ON’.
- No extra questions. End of Case 1

IJ - Case 2: Bacterial Keratitis

- MK is not specifically Bacterial Keratitis. Remember, MK consists of bacterial, fungal, acnathamoeba. So be specific to Bacterial Keratitis
- Px CL wearer, shows me a picture with paracentral (pupil-involving) approx ~2mm defect with underlying stromal infiltrate
- I described what I saw - size, location, depth
- She asked me: DDx? - CLPU, fungal, acanthamoeaba, bacterial keratitis
- How do you differentiate between CLPU and Bacterial keratitis?
    - Talked about: location, size, depth, mechanism of both diseases, AC reaction, Hypopyn

- Management?
    - Since central, with deep ulcer, even though the size was only about ~2mm, you'd refer this to a hospital for dual fortified antibacterial
        - Tobramycin 1.3% + Cefazolin 5%
    - Mechanism of these drops?
        - Protein synthesis inhibitor (30S) - Aminoglycosides + Betalactam
        - Can you prescribe these drops? - No, refer
        - How about just tobramycin?
            - You can't prescribe fortified (1.3%), but you can prescribe 0.3% as per OBA
    - Say this was not centre involving, and not too big, what else can you do?
        - Fluoroquinolone monotherapy - Nucleic synthesis Inhibitor
        - Ciprofloxacin 0.3% - q15 for 6hrs, then 30mins for rest of the day - wake px during the night too
        - Atropine 1% - ciliary spasm to minimse pain, break synichae if there is any
        - FML 0.1% - when the lesion is re-epithelised - refer to SCUT Trial
    - What other test can you do to differentiate between different microorganisms?
        - Corneal Scraping - refer
    - What else? With slit lamp..?
        - Had no clue
        - But the anwer is - CONFOCAL MICROSCOPY
    - End of Case 2.

3:20 zoster + chronic angle closure glaucoma!

1. Showed me a photo looked obvious dendritic looking super blurry :/
2. Differentials
    a. HSV keratitis
    b. Hzo keratitis
    c. Acanth
3. Differentiate hsv hzo
    a. Yapped about dendrite vs pseudodendrite
4. ok this person has skin rash respect vertical
    a. Me: yeah that's zoster x
5. Treatment - refer for oral NO TOPICAL
    a. within 72hr coz post herpetic neuralgia
6. What that and what do u do
    a. Pain! Rip and gomo - GP, dermatologist, pain specialist. Can give warm compress for skin and cool compress for eye
7. Screwed me on ORAL PAIN MEDS ???? Paracetamol ibuprofen jdkkk ah COZ THE PAIN IS BAD SO NOT MY SCOPE
8. and then asked me !!!! ???! In what countries can optoms prescribe oral antivirals ??! I was like not here mate
9. What if its stromal or uveitis
    a. Give the steroid with antiviral coverage
10. Other points: it's contagious in vesicular stage so stay away from pregnant women, also contraindicated in severe renal failure, precaution for elderly

i wanted to yap about dilating but she cut me off and Elizabeth started

1. 60 F East Asian, history of headaches after dark room smth like that + iop 20 OD 26 OS
2. What u gonna do ! Gonio.
3. No photos all verbal - PACS OD (2 quadrant no IOP rise no synechiae) and PAC OS (3 quadrant no structures seen high IOP no synechiae tho)
4. Signs of chronic angle closure - i forgot to say glaucomflekdnoops - PAS
5. Showed me oct and vf - described this from top down for each page, don't forget to comment on reliability!
6. Structure function concordance yes - so treat it !
7. Drops -PGA as long as no contraindications (lol the entire time I was like screeching PROVIDED NO CONTRAINDICATIONS- there was no paper for me to write on so they were both looking at me all amused coz my memory was cooked and I kept double checking the px conditions and meds) and LPI !! referenced the ZAP trial
8. Wanted to mention cataract surgery but forgot and wasn't prompted oops (EAGLE study)
9. Fumbled a bit around studies coz central defect so advanced ? (don't forget AGIS is for surgical options)
10. The end. Slipped out in a sweat but alive!

Both were nice tbh ! And the pace was good and they will prompt you to be on the right track coz my brain was speeedddinnnggggggg

3:40 session

- Case PXF and Traumatic Uveitis

ok so here's a full run down

Elizabeth tells me px is a 50 year old px and pulls out a picture of a pupil with pseudoexfoliative material on the lens (ur stock standard photo of PXF)
so this is LE (pxf) and RE is normal
Q: ok so what do u see

- Pseudoexfoliative material on the lens

Q: ok what else do u wanna look for when u see pxf ?

- Check angles, gonio (check pigmentation of TM and angle structures), Check for TID, check IOP, loss of pupillary ruff

Q: what are u looking for in TID?

- TID near pupillary margin

Q: ok pressures are 24mmHg LE now what do u want to do ?

- check the back of the eye and look at the discs and posterior eye health

pulls out Red free and coloured images of LE DRS photo
Q: describe what u see ?
(lowkey photo was blurry and discs looked fine for the most parts so i bsed my responses on vibes)

- temporal PPA, discs margins clear, red free shows potential inferior wedge defect ?? maybe superior wedge defect??

Q: what about the NRR

- uhhh honestly looks intact to me?? but maybe thinner a bit inferior ??

Elizabeth: lol yeh it's a bit blurry and hard to tell yeh and moves on

Q: ok what u wanna do next?
- Visual fields

*whips out visual field of LE with some dodgy af field defect points tbh
Q: explain what u see
- looking at reliability it's reliable, and there's like a superior paracentral defect ??? and maybe a inferior nasal step ??? (doesn't make sense at all to me) but i would wanna check it's repeatable defect

Q: ok so what wpuld u wanna do to confirm the central defect ?
- do a 10-2??

Q: ok so how would u like to treat
- i would refer to opthal since it's secondary glaucoma and go for SLT but i can start treatment since it's high pressures. would start on PGA

Q: ok is SLT effective ??
- uh yes according to the light trial

Q: what does light trial say about effectiveness
- 74% don't need medications after SLT

Q: ok what if the px has a history of uveitis
- ditch PGA and go to beta blocker

Q: what's CI of beta blocker
- cardiovascular issues(bradycardia etc), oral BB, respiratory problems(asthma, COPD etc)

Q: what if the px has mild asthma ?? is this an absolute contraindication or precaution ??
- uhh (got stuck in this) it's a relative contraindication ??? I would check with GP if it's ok but start them on another drug class ??? (dunno if i shouldve just said no to beta blockers for asthma all together, i don't trust me lol)

end of posterior eye case

Isabelles turn !!
ok so this 50 year old dude gets hit in the eye by a foot ball and u see this
whips out a photo of a really red eye and hypopyon and corneal oedema present
Q: what do u see
- g4 conjunctival hyperaemia, hypopyon, corneal oedema

Q: Ddx??
- Traumatic iritis? Uveitis? Q: ok say its traumatic uveitis, what's ur next steps
-
- check angles, pressures, corneas

Q: how would you check angles??
- NO GONIO!! I would check on van-herrick

Q: Why no gonio
- increase risk of bleeding and haemorrhages

Q: What would u do
- coz its got hypopyon I would refer off to opthal but can start treatment with predforte 1%

Q: why predforte? What's the preparation
- coz higher penetration with sodium acetate compared to phosphate

Q: dosage?
- Predforte 1% loading dose every 15 mins for 1-2 hours and then q1hr after that

Q: is predforte on the PBS??

- yes but only restricted to use for uveitis

Q: what else can u prescribe

- I can also give atropine 1% to help with pain and prevent synechiae and further haemorrhage — given there's no CI for it

Q: ok what if this px has a history of herpes simplex??

- prophylactic oral aciclovir from GP (didn't ask me about dosage)

Q: ok say this person went to opthal to get treated and everything settles and comes back to you, what's the prognosis ??

- you want to check for angle recession on gonio once the inflammation has died down

Q: ok so say the patient has angle recession what is the prognosis ?

- 6% of pxs with angle recession go on to develop angle recession glaucoma

**finished in 14 mins coz they ran out of questions and told me they had enough info?? (No idea if that's a good thing or not)

Overall Structure of exam

- You are presented with case history and photo of the eyes and asked to describe
- Ddx (tbh they didn't really ask me in depth about ddx, I just named ddx and they moved on- but I guess supervisor dependent lol)
- Signs and symptoms of diagnosis
- Work up - what tests you would do
- They then present you extra photos along the way (e.g. VF, fundus photos)
- Management - what drugs you would prescribe and dosage
- Know your contraindications for drugs
- Know prognosis for disease

They lowkey kinda guide you through your thought processes towards the correct answer if you don't know the answer immediately. You kinda know if you answer something wrong because they start probing towards it.
Good luck!!

Tuesday

Good luck everyone!!!

Tuesday morning

Examiners: Suki and Angelica
Case report: NTG
What did you learn
Asked me questions from feedback

Case 1: AAC
Shows me pic of one eye that's red and oedematous with dilated pupil and the other eyes fine
What tests would you do?
What are your differentials?
What's your management?
Asked me what benefit do you have in metro Sydney compared to regional??? I didn't know what she wanted… I think she wanted me to say iridotomy access to emergency care (I think this is a risk factor listed in ZAP?) other risks - strong family history, other eye AAC
The she asked what's 3 reasons why you want to do PI??? I said to reduce iop by overcoming pupil block and prevent the other eye from an acute attack… wasn't sure about the last reason

Case 2: Fungal keratitis
What's your differentials
How would you rule out your differentials
What's your diagnosis
Can you prescribe anti fungal? Said no I would refer
Would you just get them to go and wait several hours? What would you give? I said write a referral and call hospital so the px doesn't have to b e triaged so they can be seen sooner
What if the px was in rural? I said get them to drive to metro hospital
She was probing me to give the px a tx in the meantime? I said cipro 0.3% same dosage as MK
What are you worried about? Corneal perforation especially if we don't give anti fungal

What else would you be worried about? If ulcer was more than 2mm, outside central 4mm, elderly 3

10:00am Suki and Angelica
Asked about case reports: PSS and Angle recession glaucoma - what you've learned from the feedback (I think you can freestyle this part if asked - they ask targeted questions based on what you've said)

- Good to study RANZCO guidelines on whether a certain type of glaucoma is suitable for collaborative care

Case 1: AAC? - photophobia, haloes; mid dilated pupil OD, corneal haze, conj hyperaemia

- Differentials
- What workups would you do

- (IOP OD is 50mmHg, and angles are closed) - Treatment?
    - What else do you have to wait for before instilling pilocarpine, other than <40mmHg - *I couldn't answer this but it's probably ophthal approval, btw*
    - Function of Pred Forte in the treatment plan?

Case 2: Image of inferior endothelial fat KPs
- Differentials
- (Say they also have ciliary flush and conj hyperaemia): Treatment
    - (Say the patient comes back with no improvement/worsening of symptoms - follow-up?): Essentially got probed to use steroids with acetate component instead due to better corneal penetration (i.e. Pred Forte or Flarex, not FML)
- (Say their pupil's oval shaped): Treatment?
- (Think I got cut off halfway through answering the question - they probably had more questions but this is all I could remember)

10:20
Case 1: Image of mutton fat KP's (AAU)
- What do you see
- What conditions can affect the corneal endothelium?
- Say there is an egg shaped pupil, how would you manage?
- Why do you use atropine?
- How do you break synechiae?
- What would you see in posterior involvement
- What symptoms would the patient experience in intermediate involvement? What symptoms would the patient experience in posterior involvement?
- Why do you need to refer for posterior?

Case 2: Tilted disc syndrome
- Why is tilted disc syndrome difficult to distinguish for glaucoma
- What is the typical retinal appearance and VF defects in tilted disc syndrome?

Case 3: PDS
- Given a picture of a gonioscopic angle. What do you see?
- How do you differentiate PDS from PXF?
- How would you manage?

Case 4: Fuch's endothelial dystrophy
- What are the signs and symptoms?
- What are the symptoms early in the condition?
- Why do these symptoms occur (pathogenesis of the condition)?
- How would you manage?
- What is the review period?
- How do you get hypertonic saline solution?
- What other management options are there?

12pm:
NTG case report:
- What other tests would you have done for this patient?
- What is considered advanced glaucoma?
- What does mean deviation measure?
- What does pattern standard deviation measure?

- What does GHT stand for, and what does it measure?
- What do the vertical lines on the gaze tracker indicate?
- How would you determine reliability of a visual field result?
- What tests would you conduct to rule out neurological disorders?

Case 1: AAC (photo of red right eye with corneal oedema and a dilated pupil, other eye clear) :

- Describe the clinical features you see in the photo
- What condition is this?
- How would you manage this condition?
- How would you change your management if your patient vomits their Diamox tablet, as one of the symptoms of AAC?
- Why do you want to use pilocarpine?
- Contraindications of pilocarpine?

Case 2: Image of mutton fat KP - Px is a 12 year old female, hyperaemic right eye, left eye clear.

- Describe this photo
- Differentials?
- Say this is acute anterior uveitis, how would you manage this patient?
- What else would you like to ask this patient?
- Say this patient's mother mentioned that her daughter mentions pain at the joints when she runs, what systemic conditions could she have?
- What investigations would you ask from the GP?
- So you are going to initially prescribe PredForte every hour, how will you taper?

Note: I recommend reading Webber & Sharwood's article on prescribing ocular medications in children and infants: **https://doi.org/10.1080/08164622.2021.1877533.** ***Treatment for AAU in children requires co-management with paediatric ophthalmologist/rheumatologist with topical steroid/cycloplegia requiring ophthalmological supervision.***

Tuesday afternoon:

Examiners: Rebecca and Fiona

CASE 1: PXF
CASE 2: Herpes Zoster
Ahhh Ill add my questions in a bit

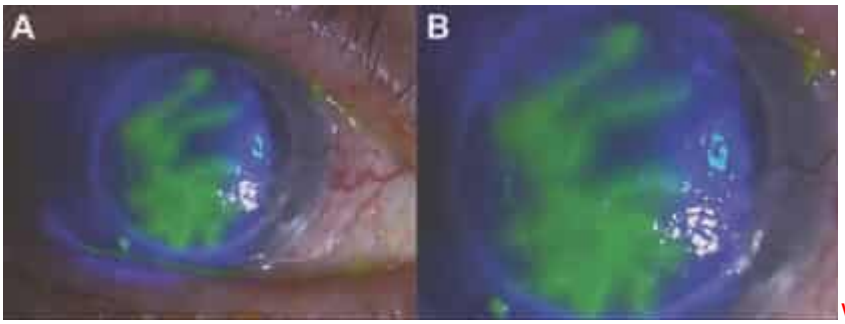
D: this arvo was fked - very, fiona

1:20pm PXF and maybe MK (not sure) - didnt ask about case reports
Case 1: shows image of very obvious dilated pupil with hoarfrost ring, no other details were provided

- Asks what I see and then asks what other signs I would see on anterior assessment
- What is the pathophysiology and mechanism of PXF

- Given this is PXF, how would you manage?
- Would you refer to ophthal? What would they do thats different from you?
- What tests would you do for each review?
- How many times can VF be billed under medicare in 1 year? I think 2 times - yep
- Since PXF has risk of fast progression, you said you would review every 3 months and do visual field. Would you get the patient to pay out of pocket or do it free?
- If the patient ends up progressing to PXFG how would you manage?
- If you gave timolol, and one of your colleagues swapped it to betaxolol, why would they have done this? Betaxol is beta-1 selective so it only targets heart and can bypass lung so asthma ok
    - Dunno lmao, told her its discontinued so i didnt learn, she said fair enough but its still on our list

Case 2: shows an image like this



wtf yikes

- Give me differentials
    - HSV, HZO, abrasion, acantho (then got cut off, was gonna say MK)
- How would you differentiate HSV and HZO?
    - Dendrites vs pseudodendrites, terminal bulbs, facial rash, hutchinson
- Is there any branching here?
    - Kinda? Not too sure
- If this was HSV what would your treatment be?
    - Aciclo 3% 5x/day + OL
- Why see them next day?
    - To see if there are any worsening of symptoms or signs
- Okay, lets say their symptoms worsen next day and now you find an overlying infiltrate next to that lesion what would you do?
    - Suspect different microbe? Possibly bacterial? - like surely, if worsening symptoms and sudden infiltrate?
- Okay, if this was bacterial keratitis how would you manage?
    - Cipro 0.3% quarterly for 1st 6 hr, 30min rest of day, review next day
    - Atropine 1% bid
- Okay this is all we have for now, right on time.
- Fiona says at the end - leave now run away as far as you can before we change our minds ????? WHAT XD does that mean i did well or bad??? Im scared
    - This whole second case was confusing, dunno if it was HSV or she led me down the wrong tract (MK) or it was MK

Other thoughts - acanth?? Can also present with pseudodendrites - yeah i brought it up as a ddx but fiona just completely ignored it, it was non-stop questioning, i couldnt even ask any questions like IOP for the PXF

Yikes ! highkey the same thing happened to me like they have their own direction and cut you off

Severe anterior bleph + Angle recession

3pm
Traumatic uveitis +neurological
Case 1 neurological
They just pulled out the ONE EYE visual field directly, ask me what defect it is… I was like: WTH?one eye
After describing the VF, ask me which part of visual pathway is damage (Totally cant answer - silence during the whole room, dont ask me what imaging looks like, PLS everyone review this)
They let me to say each part of visual pathway will cause what VF defect…(WTF???)

Extra: ask me the glaucoma drug - precaution and contraindication.

Case 2 Traumatic uveitis (similar questions she asked above)
Describe the appearance of the anterior eye
The differential diagnose based on the pic, justify the reason
Management: specific name of drug, dosage etc
The mechanism of the drug
Time OUT

- Hey guys. Are we allowed to begin management for secondary glaucomas like pdg and pxg? Or do we need to just refer because they're recommended as not being suitable for collaborative care as per the RANZCO guidelines? I would say this first! And then let them prompt you on what the ophthal would most likely do which is ur drops/SLT

Ok, thanks!

7. Collaborative Care for Glaucoma: Recommendations
   - The accuracy and reliability of data collection is paramount. No optometrist should engage in glaucoma screening or monitoring unless they are able to perform clinical examination at least to the required standard of the appropriate regulator AND perform accurate and reliable measurements with the equipment available in the practice.
   - Patients at high risk of visual loss from glaucoma should not be part of a collaborative care scheme and should be managed by an ophthalmologist.
   - These include patients with:
     - Complex ocular pathology/secondary glaucoma (except for PXF or PDS as included above)
     - History of eye trauma with established glaucoma
     - Monocular patients with glaucoma
     - Patients who have undergone multiple ocular surgeries
     - Patients with advanced glaucoma (stable or unstable)
       - unstable glaucoma
       - acutely raised or very high (>35 mmHg) IOP need

For anyone who wants to read what we do re: referring complex cases of glaucoma ^^

For ‘complex ocular pathology/secondary glauc’ we refer w/o Mx ourselves… but for PDS/PXF we can initiate Mx.

:(sounds like tuesday afternoon was super rough
Hope everyone can have a good meal tonight and just rest <3
If anyone wants to talk about it feel free to dm me - angela z (my exam was monday so i’m recovered now)

Wednesday

Good luck everyone!!!

Im shitting it ahhhhh

Me tooooo

Cryyyy

MORNING SUPS: Lily & Elizabeth (chill)

9AM: OHT & HSV/HZO

Anterior case report questions on AAU - what did you learn (don't dig yourself a hole! Mention smth you know good info on) - mentioned important to check posterior for intermediate and posterior uveitis, Lily then asked what other conditions can have posterior involvement I said HSV/HZO that became my case 1 :)

Case 1: HSV Epithelial Keratitis/HZO - No case images (Lily)

1. Differentiate between HSV and HZO - talked abt skin rashed which she latched onto and asked about pathophysiology of both e.g. where they both lay dormant and how they reactivate
2. Px presents with epithelial dendrite - what do you want to do? Go through your full workup make sure to mention posterior check with DFE
3. Lily asked why do I want to check motilities if I'm suspecting HSV keratitis? If differentiating between HSV/HZO, HZO can present with CN palsies while HSV doesn't usually (not sure if this is right)
4. Management and review of HSV Epithelial Keratitis
5. Px comes back 1 week review with stromal haze, epithelium healed - management?
6. Lily prompts me to talk about recurrences and asks what type of patients can have recurrences - immunocompromised → what type of conditions? I mind blanked she said we can come back to it (never did teehee)
7. What will you do if your patient keeps having recurrent attacks? Send to ophthal for oral prophylaxis

*No CI or SEs asked i just mentioned along the way

Case 2: OHT (Elizabeth)

1. Shows left eye disc photo no issues normal looking disc and fundus - 45 Male presents with 29mmHg OU what do you want to do? Full glaucoma workup, rule out secondaries (realised i should have asked for RE fundus rip)
2. No secondaries, open angles, borderline low CCTs - what do you want to do? Mentioned drops, she cut me off asking is that the only thing you can do? No SLT
3. Went through SLT benefits - will directly ask for studies
4. Went through drops - PGA, what would you explain to px? Go through SEs - didn't directly ask for CIs so i mentioned along the way
5. Okay px doesn't want drops so they had SLT when do you review them - couldn't remember if it was 2-4 weeks or 6-8weeks i said 4-8 weeks LOL
6. SLT done but px has residual high IOPs, what now? Start BB timolol, but mentioned if active young male then don't want to give it bc bradycardia + check other CIs px just didn't want cosmetic side effects bc young
7. Who can you talk to to check if okay to give timolol to patient - ophthal who did SLT and GP

8. Okay px doesn't want to take drops, so only SLT so far, but pressures are still high - what can you tell them about the dangers of having high pressures (other than glaucoma)? Mindblanked on this one and said some random stuff yikes
9. Then they let me go

9:20am

Anterior
Shield ulcer ddx? (not just VKC) - what else? - I said GPC - she then prompted me to say AKC

Case went into AKC
- Corneal signs youll see: I said persistent epithelial erosions/SPK always bilateral also skin eczema and lesions. (I missed to say NV)
Not asked for treatment of AKC - immediately went into VKC case but got asked in such a weird order lol
- Asked sx and symptoms
- Pathophysiology
- If there was trantas dots but less symptoms what to do - FML 0.1 qid (fast taper if less than week slow taper if longer)
- Trantas dots comes back what to do - immumo injections from GP - she said ok
- Now no trantas dots but symptoms - olopatadine 0.1% bid mentioned AH helps with itch

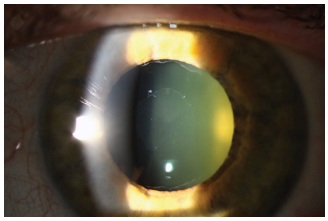
Posterior

- NTG OD was perf OS was inferior RNFL defect
- Asian female 15mmHg IOP OS
- Spoke about ddx neurological bc very asymmetry
- What fields to do 24-2 and 10-2 bc NTG paracentral defects
- Workup for neuro: red cap test, RAPD, MRI forgot what else I said lol (forgot to mention 30-2 neuro)
- Went through all drug classes for treating IOP - PGA, BB, CAI, AA, combo: their % IOP lowering when to review each class and side effects and opthal referral

10:20am: AAU & PXG (no case report Qs)
Case 1: AAU (no pictures, just verbal) - Lily
1. Px presents with Grade 3 cells in the anterior chamber. What do you do? - started with H&S, onset, pain, has it happened before, ocular history, PMHx, medications
2. What questions in particular would you ask regarding their systemic health? -
3. You mentioned asking the patient if this has happened before. Why do you want to know that?
4. What can cause Grade 3 cells in the AC? - acute anterior uveitis, started listing out causes of AAU then she cut me off…, HSV, injury, EKC
5. Do you expect Grade 3 cells in EKC? - no
6. What else do you want to check? - VA, slit lamp and NaFl for any corneal defects, posterior involvement

7. What woud you see if the posterior is involved? How would you check? - DFE, vitritis, retinitis, choroiditis
8. When would you dilate? Would you dilate every single patient? -
9. Assuming no posterior involvement, what do you do then? - treat with PredForte 1% q1h, then 4x/day

Case 2: PXG - Elizabeth
1. Px presents for routine exam. IOP is R 19, L 24. What do you do? - H&S, family history of glaucoma, any migraine, Raynaud's phenomenon, sleep apnoea, entrance testing, pachymetry
2. What else do you want to do? - slit lamp and fundoscopy to check the disc appearance
3. Before you check posterior? - check anterior, check if angles are open, pigmentation, transillumination defects
4. Gives photo of ant eye showing PXF material on lens. What do you see? - PXF material on anterior lens surface



5. Can you describe how this happens? -
6. Shows pic of OCT RNFL printout. What do you see? - inf and sup thinning in LE
7. VF shows defects (no picture, just told verbally). What do you do? - start treatment with PGA eg latanoprost or BB such as timolol
8. Px has hypertension. Which would you go for? - latanoprost
9. When do you want to see them next? - 6-8 weeks followup, want IOP to be lowered by 25-30%

11:40
Case 1: Allergies
1. Explain/run through treatment for allergies
a. Asked about all drops, concentrations. Mechanisms (histamines or MCS). Onset for effect to take place. ASKED FOR A BRAND NAME. Then asked again what is in the drug - made me question myself saying "Are you sure?"
b. What can be done for immediate relief?
c. How can shield ulcers be formed? How is it different to an infectious ulcer? Other than due to papillae/mechanical how can shield ulcers be formed?

Case 2: NTG
1. Was given basic px information and IOP and RNFL OCT scan. Went through other tests to do (gonio, red cap, colour test, water drinking test). Given VF it was a paracentral defect MD like -6. Something.
a. Asked why D15 should be used and not Ishihara
b. Asked what to rule out.
c. How would you manage? PGA. How does PGA work to lower eye pressure? What if the patient is noncompliant and says "I sometimes missed days, other days I put in two drops?" What do you tell or advise the patient?

d. Anything else you wanna do other than optometrist management?
e.

WEDNESDAY AFTERNOON
SUPS: Fiona and Anna

I would recommend quick read tfos dew iiii that kate sent in optom'25 chat just in case <3 (coz fiona loves her dry eye)

AAC
-signs + symptoms
-tx
- aetiology
- differential: possner schlossman syndrome
- how to differentiate
- tx for PCS

PRESEPTUAL CELLULITIS
-tx
-VA, RAPD, EOM, Proptosis
- systemic associations
- Tx: oral anitbiotic - which one
- how would tx change if orbital

Extra qn cos i had time lol: ciclosporin - tell me about it (said PBS stuff)

Fiona thumbs up and smiled then told me to leave LOL

2:00PM: NO CASE REPORTS
VKC but led to steroid induced response/glaucoma in a kid - 17 year old, been using steroid long term dexamethasone beyond practitioner advice
What is Horners Trantas dots
Differentiate SAC vs VKC
What type of reaction is VKC
What is a steroid response - how does does work
Who is more prone to steroid responses
% of high responders vs what is considered a high response
Name two steroids with the highest penetrance
How would you taper
When would you initiate tx and what circumstances would you keep the steroid
If they were using steroids short term would you commence tx
when would you review the patient - would it be ongoing monitoring (yep don't want to get glaucoma + risk with future steroid use)

CI and pre cautions of different glaucoma classes

HSV? I think she was trying to get corneal abrasion! But she just stuck with herpes

Not sure Fiona kept asking for differentials and just said ok lets go with HSV
In my differential she kept asking me so I slipped and said corneal ulcer - she stuck onto this and asked what the difference between corneal ulcer vs MK (MK you get corneal ulcer + infiltrate)
How do you differentiate HSV vs HZV
What type of people do you expect in HSV
What treatment would you give - ointment vs oral
How does HZV treatment differ + dosages
What are you worried about in HZV

You are prescribing Ciclosporin for your patient
What do you do - talked about PBS restriction prescribing -> workup for dry eyes
What is the dosage/brand name
When do you expect it to work
Do you expect signs or symptoms to reduce first
Side effects
What do you tell the patient - didn't really go through this so just said the side effects and that it will go away with more use
How many repeats can you get with the PBS?
How do they insert the drops - i didnt know but apparently closed eye in punctal area and px slowly opens eye
When do they insert the drops - i just said morning and night IDK i was just making it up

But honestly they seemed happy. Anna was really nice and bam bam. Super intimidating but pre rapid fire and you know more than you think!!! Also they kind of guide you to the answer if you are off track or something theyll get you to answer again

2:40 and 3:00 Angle recession glaucoma + Marginal keratitis
(we have same case lol)
Anna: Angle recession glaucoma
40M, dull pain in the RE. IOP R 25 L 19. VA 6/6
What do you do next? Gonio
Shows a gonio photo with widening of CBB and there is a pigmented spot on the Schwabels line?
What did you see? Widening of CBB
What else? There is a dot above Schwabels line **What is that?** pigment clumps
How do you know is widening? Compared to the other eye

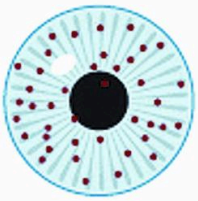
Risk of glaucoma? >180 degree of angle recession
What will you ask? When is the injury and what happened?
She said 8 years ago there is a blunt trauma and showed a hyphema photo
Grade hyphema: 1 because less than ⅓
What grade has a higher risk of developing glaucoma? I said >50% (Grade 2)
Can you do gonio? NO

What else can cause hyphema apart from blunt trauma? Heavy lifting, blood thinners and she asked what else and idk and she said we can come back later (which we never)

What tx for hyphema? Bed rest, 45 deg of head tilt blah blah blah
What test you will do in angle recession? VF, OCT, Fundo, CCT
CCT is 515um, so what? Underestimate IOP, higher risk of glaucoma
How do know test IOP fluctuation? I said water drinking test but she asked for something more clinical? Idk and i said ask them to comeback at another time lol
She then said the IOP fluctuate from 21mmHg to 28mmHg then what wil you do, I don't understand and said something like 28mmHg to calculate target IOP?
Then we move on and ask management. I said refer to ophthal in 4m (I don't think you actually need at this stage cuz she didn't say any VF defect/RNFL or CDR??) then silence and I said start timolol 0.5%
Asked BB contraindication
Said PGA is first line and asked why I gave BB
Asked PGA MoA
Ask pilocarpine MoA

Fiona: shows a 144p pic of corneal infiltrate in superior
What do you see
DDx of corneal infiltrate (Adenoviral keratoconjunctivitis, Thygeson superficial punctate keratitis, CLPU, marginal keratitis)
Management of Adenoviral keratoconjunctiviits educate px contagious, cold compress, PF lubricants
What will you do if there is staining on top of infiltrate
MX of marginal keratitis

Ok is 20 mins u can go now
Said bye and ran out of the room :):)



Microhyphema
Circulating red blood cells



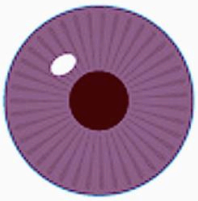
Grade I
≤ 1/3 anterior chamber vol.



Grade II
1/3 - 1/2 anterior chamber vol.



Grade III
> 1/2 anterior chamber vol.



Grade IV
Total anterior chamber vol.
"eight ball hyphema"

Thursday

Good luck everyone!!!

Morning sups: Suki + Daisy

SUKI AND DAISY

9:20am SUKI AND DAISY
asked me a lot of questions about my case reports
My first case report was about hordeolums so all of that stuff and then it went into preseptal cellulitis
What would you prescribe for preseptal?
Why antibiotics r better than topical?
Then got given a photo of MK
What would you ask the patient?
What's your work up?
What are your differentials?
What's your treatment?
If they come back in 48hrs and no change consider referral to hospital for fortified
I think she wanted me to say fortified first because the lesion was biggish

Posterior was literally just my case report NTG
Mechanisms behind NTG
What questions would you ask in history? Risk factors to look out for?
Had to go through every single drug class and contraindications
What drop for each class and what dosage
Why is Brimionidine a good option? Neuroprotective effect which is good for NTG

10;00am
Case Reports + Bacterial Keratitis

1. What did you learn in your case report? - went on about differentiation between VKC and AKC and management. Went on for a while and got asked some very specific things lol
2. Asked about onset of mast cell stabilisers? - highkey had no clue
3. Got shown pic of bacterial keratitis (OK wearer). What differentials? - Fungal, Marginal, Bacterial
4. Management? - Cipro
5. What if it doesn't resolve and symptoms get worse? - hospital dual fortified therapy
6. What drugs, dosage, concentration?
7. Why dual fortified? - full spectrum coverage

Case Report + AAC

1. Got asked about case report on NTG and typical age range of NTG? - highkey did no remember
2. Got asked about CCT and IOPs - pressure variations and risk factors
3. Dry eye and PGAs or other conditions - went on about lumigan PF instead of xalatan.
4. Side effects of PGAs

5. What if patient comes in with acute red eye, lots of pain and photophobia? Differentials? - uveitis's and AAC
6. What test to confirm which one is its? And what do you expect? - IOP super high in AAC
7. AAC first aid? - full explanation.

9:40

1. Photo of extensive dendrites

Top differentials HSV, HZO, Acanthamoeba, explain for and against
Focus on HZO
Signs—dug great hole, suki asked how does hutchinson sign develop and how does it related to the trigeminal nerve —--> I AM DEAD
Asked about skin lesions where is it on the face
Treatment: refer opthal asap , better within 72 hours —> why (i didn't get it either), shd be shorten duration and pain
Asked about how is postherpetic neuralgic pain developed (this hole is getting bigger)
Asked about What oral treatment will opthal give (Acilovier oral 800mg 5x/day, i said also to refer to GP and pain specialist → she side eye me 'so u r gonna send her to 3 different places huh?>
OMG i forgot to say do not physically contact immunocompromised preg women, isolation

2. (Case) POAG atypical young male

Prognosis of glaucoma in young px → not so good, need to use drops the rest of his life
If there is a 30yr and a 80yr POAG, what is their difference → i talked about maybe cataract extraction in 80 yr and she asked about the percentage of IOP i completely mind blanked. Not sure, i said both review slightly closer like 6 mo
What is differential → PDS, young and myopic

3. Move on to PDS

Signs - krukenburg spindle, pigment in TM, iris TID
Symptoms - HA in active movement
Treatment - SLT/ PGA then BB then combo

4. AAC

First aid

Good luck guys, i fked up so much, let me emo for a while

10:20
Anterior eye case report questions (Corneal abrasion - spent a long time on this)
Then traumatic hyphema

- What do you see?
- How would you treat it? What else can you do besides bed rest with 45º tilt
- What other signs might be involved here?
- Went into uveitis and its management

Posterior eye case report (PXF)

- What is pathogenesis of PXF? And its prognosis?
- What other organs does PXF affect?
- What study talks about the risk of progression of PXF compared to others?

- What's the difference between PXF and PDS in signs/symptoms?
- What typical demographic would you see in each?
- What option would you give a patient if two different lowering-IOP drops were given (i.e. combo)?

Neurological case

- Gave a visual field with a right eye nasal hemianopia (respecting the vertical midline)
- Say this was the same in the other eye (i.e. homonymous), what do you suspect?
- Where would the compression or lesion be?
- What if the visual field defect was bitemporal?
- What sort of diseases would occur at the optic chiasm?

AAC case

- What is the treatment for AAC?
- When to use pilocarpine and when not to?

Hello guys !! Last spurt !! good luck !! hopefully this helps ((': im ngl these do seem like repeats from the previous days I would recommend going back to whatever say the sup has examined before if ur worried

11:40

Anteiror eye Case report questions (I did AAU)

- What did you learn? talked about learning how to treat AAU in a clinical context with the steroids and cycloplegia being used to stabilise the BAB, pain relief, preventing posterior synechiae (planted this cos ik in my report I didnt rly talk about the breaking of the synechiae MWAHAHAH)
- so what did you do to break the synechiae (MWAHAHAH)
    - atropine 1%
    - phenylphrine 2.5%
    - pledget with phenylphrine 2.5% (anaesthetic -> soak da pledget -> put in inferior conjunctiva fornix; forgot to say how long but 15 minutes)
    - and it didn't break? nah it didnt,,,
    - what else could you have done? phenylphrine 10% but we would have to measure blood pressure before due to its cardiovascular effects
    - when else do u need chrck BP? lowkry got confused but she just wanted me to say after instillation LOL im a dumbass
    - what happens with phenylphrine 10%? hypertension!

Anterior eye: MK im ngl this lowkey is a blur

- shows me picture of a FAT ulcer; not exactly visual axis involving but she is large and stromal glowing
- What are ur ddx and what rules them out -> acanthamoeba, Fungal, Bacterial
- whats ur management -> i referred straight up for dual therapy gentamycin 1.5/tobramycin 1.3 + cephalosporin 5%
- what if the hospo cant take em? HUH start them on ciprofloxamin q15 first 6 hours, q30 rest of the day waking and asleep, hourly 2nd day with review to ensure its not worsening, 4 times per day until reepithelisation (Which is wrong its every 4 hours IM A DUMBASS) and had to rly clarify the timing of the drops
- I lowks dont know what happened here but it went into like oh what if its worse oh what if its same symptoms in circles? I just was like yo if its the same im not expecting thr symptoms to get better the next day right im expecting it to get better as

the week goes (??) if its worse as I said before we would go for dual fortified cos antibiotic coverage u know then dug myself a hole with what drugs have what coverage
- i said cipro is broad spectrum with greater coverage for gram negative, and I had to go thru what drugs are gram negative (fluoroquinolones, aminoglycosides) what were gram positive and if within optpmetry scope (macrolides, tetracyclines, penicillins, cephalosporin BUT needs compounded for macrolides and tetracyclines, otherwise oral and suki seemed happy with that)
- when woukd u go for dual fortified ? 2mm, involvinf ⅓ of the cornea, older age
- why? worse prognosis
- wdym worse prognosis? greater chance of scarring and causing worse visual outcomes? suki stopped there

Glaucoma case report questions (I did POAG)
- Age range for POAG; i said 50 then Daisy looked surprised i said 50 so I changed to 70 and she seemed more surprised and I was like uhhh and shes like its fine LMFAO IM DEAD
- “You ruled out many secondary Glaucoma here can you tell me thr diff between pxf and pds”
    - age and demographic; PDS young male myopes VS PXF older female
    - More systemic associations with PXF
    - TIS PDS mid peripheral VS PXF pupil margin
    - gonio more homogeneous dense pigmentation in PDS and more splotchy and Sampolesis line in PXF
    - obv krukenberg spindle pds and white pseudoexfoliative material on da lens etc. Daisy was happy with all of this and moved on
- smth about the fact we used pga in one eye; what are the considerations
    - go thru the cosmetic effects -> Daisy asked oh so the patient was chill with that and I was like yeah esp when we explained the superior iop reduction we can get with pgas
- if they have rly bad dry eye after the drops what can u do
    - ocular lubricants 10-15 minutes before or after
    - lumigan pf
    - what else ? not drops? SLT ?

Posterior eye: AAC

Daisy shows me a picture of where there is very intense corneal oedema, dilated pupil, conjunctiva hypermedia in the right eye ONLY
- tentative diagnosis or ddx? just listed them out -> AAC, AAU
- what tests could you do to rule them out? IOP -> would be very elevated in AAC compared to AAU (esp. when acutely there is a reduction in IOP with ciliary body inflammation), also mentioned the pupil being more dilated in AAC, i forgot what else I said LMFAO
- what would you like to do?
    - Need to contact ophthal or hospital emergency department for LPI as definitive treatment
    - in the meantime first aid!
    - diamox but need to liase with ophthal/pharmacist (said provided there are no CI but they didnt rly ask me for them)
    - timolol 0.5 -> apraclonidine 0.5% -> brinzolamide 1% 2 minute intervals

    - prednisolone acetate 1% if red and inflamed which it is lol q15 first hour then hourly thereafter
    - pilocarpinr 2% provided that the iop has reduced -> Daisy interjects and asks what level it needs to be reduced to and I say 40mmHg
    - Why? iris ischaemia
    - suki interjects and asks why that happens and im like ? huh ? the high iop?
    - and shes like yeah but like,,, ok whay does ischaemia mean and im like reduced vascular perfusion and shes likr which means ? and in like reduced blood flow ??? and shes like yep ok and left it there while daisy was like nah im pre happy with what I have to kinda cut her off LMFAO thanks daisy for saving me
- idk if this helps BEST OF LUCK TO AFTERNOON SESSION YOU GOT THIS IN THE BAG 🫶

So basically just my case reports yall

Afternoon sups: surely its not fiona again right….
I hope so I have 1pm session so I will let y'all know :)
ITS LILY AND FIONA STAPLETON
I GOT AACG AND Acute Anterior uveitis

Fiona

1. Picture of female 65yo os mid dilated pupil, red eye iop 55 in os
2. Asked ddx
3. Whats the difference in signs between posner schlossman and AACG
4. What do you want to do
5. Ok you have ur final diagnosis what are your first steps
6. What if they are contraindicated to taking oral diamox? What do you do??
    a. Ophthal/emergency would most likely give them IV mannitol
    b. Contraindications to diamox is severe renal disease, endothelial cell dysfunction, sulfonamide allergy
7. Asked me all the contraindications and side effects of beta blocker, alpha agonist, and CAI
    a. Asked me if they are taking a systemic beta blocker would you still give timolol: I said yes as it is not an absolute contraindication but the IOP lowering effect is not as much and increased risk of systemic effects so its a precaution
    b. Asked me what if they are taking MAOIs or they have renal disease, since its just a one off would you still give them alpha agonists?. I was like NO because it's an absolute contraindication as it can cause a hypertensive crisis
    c. She was trying to gaslight me into saying something wrong
8. Ok is her g2+ cataract clinically significant in your treatment?
    a. I said no because it is still mild and not hypermature but the ophthalmologist might consider CLE on top of LPI to open up the angle
9. What are the mechanisms of angle closure?
    a. Talked about precipitating drugs, dim room
    b. Pupillary block

    c. Plateau iris configuration
    d. Neovascularisation
10. What is it called when the lens causes an angle closure: i said pupillary block but the correct answer is phacomorphic angle closure
11. I said I would give predforte q 15 first hr and hourly thereafter and ophthal before instilling pilocarpine
    a. why predforte: to reduce inflammation
    b. why pilocarpine: to relieve pupillary block

Lily
Ok lets say this eye is a uveitic eye now, what do you do OS g3+ cells

- What is your workup?
    - Has this ever happened before (checking for reccurence)
    - quick systemic screening. why? rule out HLA-B27
- What are your differentials?
    - need to classify granulomatous vs non- granulomatous, dfe to rule out intermediate and posterior uveitis
    - rule out herpetic uveitis
    - keratitis, episcleritis, scleritis
- What is the difference between acute anterior uveitis and AACG?
    - mid-dilated pupil vs miotic pupil
    - posterior synechiae
    - IOP would not be super increased unless trabeculitis
- You said you want to give prednefrin forte AND atropine why prescribe both phenylephrine and atropine. Are they the same class?
    - no one works on the sphincter muscle and one works on the dilator muscles so they work synergistically

1:20PM
SAME CASE AS ABOVE - no diff

1:40pm (fiona & lily)
my topics;
angle recession glaucoma

- general work up for glaucoma
- review period for past trauma
- mechanism behind IOP increase
- all contraindications of glaucoma drops
    - which one is contraindicated in angle recession (pilocarpine)

lesion in the primary visual cortex

- left hemaniopia in the right eye, asked for differentials
- then said now imagine its like this in both eyes
- what is that
- how is that different from lesions in the optic radiation
- and how might that happen? (tumour, infarction, meningitis)

blepharitis

- how to DDx between blepharitis and demodex?
- how to treat?

- how to apply chlorsig ointment? (dont get tricked and say lower fornix, i said this and fiona gave me a chance to correct myself - apply it to the eyelids using a cotton bud)
- if chloramphenicol contraindicated, what other ointment can you use?
    - i wasn't sure but time ran out and fiona asked me two final questions
    - mode of action of aminoglycosides?
    - how is it different to chloramphenicol

the end
SIDE NOTE: i asked them how i did and they gave a cheeky smile and said they cant tell me. they'll have a meeting and NEXT TUESDAY we will all find out how we went. good luck to all

2pm- Vauge question about glaucoma, angle recession, pdf, pxf, visual field homonymous Hemianopia, adenoviral keratoconjunctivitis. It went so fast

Supplementary exams

Supplementary Thera Orals

Anyone know when will we know the results?

Wednesday

SOMEONE TYPE PLEASE
YES PLEASE
💀

14:20 - Lily and Fiona
(I had AAU and AAC for my first attempt - only got 1 case of AAU this time around. Might have just ran out of time. Yes it was the whole 20 minutes lol. Feedback was mainly on AAU, but I stuffed up the AAC treatment plan slightly back then. They didn't ask anything about it during the supp)
(I'm not sure if they actually ask about the same diseases you've had on your first try. I wouldn't bet on it, but also IMO they asked pretty in-depth Qs so idk if blanket prepping for all the diseases works)
*Did they tell you how you went or when the results will be out??
(Also I can't actually remember all the Qs they asked sorry)
Case 1 - AAU (Lily showed me a blurry photo with a constricted pupil and potentially AC cells in the slit lamp reflection)

- What do you see?
- What do you suspect?
- What other causes can it be? (something along those lines sry)
    - Might've been asking for AAU causes? HLA-B27, PSS, Fuch's, yada yada?
- What other tests would you do then?
- Aetiology of AAU???? I think?

- (Then shows me another photo of giga irregular pupil after dilation)
    - What this, what do? - break synechiae.
- What's the mechanism of action of Phenyl?
- What are the contraindications of Phenyl?
- What's the mechanism of action of Cyclopentolate?
- Okay what treatment would you give for AAU then
    - Say there's no more Pred Forte, what then → No more Flarex, what then → Dexamethasone
- When would you review? (I said a day later)
    - What would you expect in a day? (I just said hopefully reduction in cells and flare, Lily went would you expect cells to decrease in just a day? I got shook and said I'd expect at least no worsening in AC cells)
    - What else do you expect, why would you review in a day?
        - Think she wanted me to talk about IOP rise as a steroid side effect
    - How much of an IOP rise would you then do something about it? (something along those lines)
        - I don't actually know, unfortunately

        - What would you prescribe to lower IOPs?
            - I said timolol or alphagan
        - What if Timolol isn't working well enough?
            - I got caught between switching to Alphagan vs taking a combo drug

- (Fiona takes over)
- When would you prescribe topical steroid vs topical NSAID? E.g. what conditions?
- When would you prescribe topical NSAID in a collaborative care setting?
    - I had zero clue and she gave me an example of cataract surgery
- What drops would you prescribe post-cat surgery?
- Circles back to mechanism of actions of Phenyl and Cyclopentolate. She did a lot of prompting.
- Think that was it?

Thurs people - Please type in what you get tomorrow
Good luck!!

Thursday

Examiners: Angelica and Lily
Was it same cases as first exam? - one same case, one new

Cases: HZO, Bacterial Keratitis, PXF, PDS

- Same cases as first round but added new cases too
- Will ask what you learnt from the first time you did your orals (your chance to correct yourself from last time)
    - How many new cases did you get asked? 2
    - Which cases were new for you? Bacterial keratitis and PDS
- Felt like it was more in depth than the first orals :OOOO
- Was your feedback mainly on HZO or PXF? Or both? I was not expecting 2 new cases :(
- Me neither :(It was pretty even between both
- Did yours take the whole 20 minutes? Im assuming it did. You got asked so many questions oh no, were you able to answer all these questions? this looks even harder than the first orals - defs not or like I took a really long time to think of an answer
- Did they prompt you like in the first orals? Or they more harsh this time? Oh nooo They will prompt you… I would say they were not more harsh tho? Probably the same… actually I think I was more intimidated this time
- It felt so long ahhh but lowkey I was blanking so much
- Were any of these questions repeated from the initial oral? Or they just ask new questions?
- HZO
    - What's your management?
    - What's the difference between your management vs GP vs ophthalmologist?
    - What are your differentials?
    - How do you differentiate between presentation of HZO vs acanthamoeba?

    - Why do you want to treat with aciclovir? What do you expect it to do?
    - Can you describe what kind of rash you would expect?
- Bacterial Keratitis
    - How do you differentiate it from HZO?
    - How do you manage it?
    - Why ciprofloxacin?
    - What if you can't use ciprofloxacin?
    - What is the mechanism of action of ciprofloxacin?
    - What is the mechanism of action of gentamicin?
    - What is the mechanism of action of cephazolin?
- PXF
    - Whats the difference between how you would manage it vs the ophthalmologist
    - What are the 4 legal requirements for referring glaucoma?
    - How would you find out these requirements?
    - How would you manage this condition?
    - What is the mechanism of action of prostaglandins?
    - What are the contraindications of prostaglandins?
- PDS
    - Gave me a gonio image - pigmented trabecular meshwork and possibly angle recession??
    - What do you see?
    - What are your differentials?
    - What questions would you ask in your history to differentiate PDS and angle recession?
    - What are symptoms a px with PDS would experience?
    - What is your management for PDS?
    - What if they can't use drops what can you do?
    - What surgery options are available and rank based on which one you would consider first?
    - What is the mechanism behind these surgical options?

WTH looks so in depth

Cases: AAU & AAC

- What did you learn from your first oral exam?
    - Always dilate acute anterior uveitis to check for poster involvement
    - If posterior is involved then refer to ophthalmologist because optometrists cant manage this
- What would you see if there is posterior uveitis?
    - vitiritis, vitreous haze, vitreous cells, snowbanking, snowballs, vitreous haemorrhage, retinal haemorrhage, exudates esp around the vessels,
- Why cant optometrists manage poster uveitis?
    - topical steroids cannot penetrate through to the posterior eye
- So there is no posterior involvement, how do you manage?
    - predforte 1% loading dose every 15 min forst first hour then q1h after that, follow up next day

    - if improvement by 2 steps on the SUN grading scale then can reduce to qid until inflammation has resolved, then do a taper
    - Depending on how long theyve been on steroids for can do long or short taper
- With uveitis do you expect them to do short taper?
  no usually takes a long time to resolve, so would do long taper. Qid for 1 week, tid for q week, bid for 1 week, then once daily for one week
- Last time you said dont dilate because the iop will increase. Do you still think this? When do you not dilate? Contraindications for dilating.
    - No. Dilating is only a problem if the angles are closed.
- Anything else?
    - idk
- What do you expect iop to be like with uveitis?
    - Low because inflammation causes damage to ciliary body which means it doesnt produce as much aqueous and therefore iop is low
    - can also be high if inflammatory cells clog the ™
    - synechiae blocking the aqueous flow
- How to break synechiae?
    - atropine 1% bid
- What else?
    - phenylephrine 2.5% or 10%
- Why use both?
    - phenylphrine is stronger
    - diff MoA so works better to break the synechiae
- Px comes back the next day and IOP is increased. What do you do?
    - refer to ophthal
- What do you expect the ophthal to do?
    - change treatment, maybe to nsaid
- Can you prescribe nsaid? (Yes) Then why refer. (I guess I can prescribe) Whats the problem with nsaid?
    - Not strong enough, wont treat the uveitis
- So what else can you do?
    - iop lowering drop, timolol 0.5%

AAC

- Difference between chronic angle closure and acute angle closure
    - One is chronic, one is acute??
    - AAC iop is much higher 40, 50s or more
    - symptoms of nausea, vomiting, and haloes around lights for AAC
- Causes of chronic angle closure? Kept probing me for what else but idk what else
    - Physiological narrow angles
- How would you check for angle closure?
    - Gonio, ptm not visible for more than 270 degrees
- If your patient comes in with acute angle closure, what can you do?
    - Start first aid. Diamox 500mg oral. Optom cant prescribe orals so need to call doctor or pharmacy to get the prescription
    - Beta blocker eg timolol 0.5%
    - Alpha agonist eg brimonidine 0.5%, 2 mins later

    - CAI eg brinzolamide 1%
    - Then pilocarpine 2.5% with ophthal permission
- Why do you need to call the ophthalmologist to give pilocarpine? When can you not do pilocarpine?
    - Pupil block mechanism??
    - If px cant get to the ophthal fast enough
- Why cant you give pilocarpine if px cant get to ophthal fast enough?
    - Nvm i take that back
- What else can you do for a AAC px?
    - pain management, cycloplegic such as atropine - idk why I said this cos its actually contraindicated :(((
    - Also paracetamol or ibuprofen for pain management
- What can you give if the eye is red and inflammed? (They probed me a lot)
    - Topical steroid such as PredForte
- What is the ultimate goal for referring to opgthal? What will they do?
    - LPI
- How does LPI help?
    - alternate pathway for aqueous outflow, reduces IOP

2 cases: AAC + allergy work up - not the same cases and didn't get asked about my first oral (first oral got PDS, fungal keratitis, AKC)

Allergy

1) Allergy work up/ steps - palliative care, each type of drug (+brand names), MOA
2) Really dug in and asked when and why I would use each type of drug
3) Somehow got onto shield ulcers - what causes it, whole work up, told them Cameron scale grading and mast cell+steroid+prophylactic AB+ refer to ophthal for debride + AMT
4) Asked how SAC presents other than ocular presentation - I got stuck, I think they were looking for like runny nose/cough etc?? Idk

AAC

1) showed pic of red oedmateous eye and said sx of pain/redness starting that day
2) Asked work up - H&S, VA, pupils, etc, said gonio + Oct probs not gonna work
3) Were probing for more but idk what
4) Management
    a) Talked about diamox and how to refer for IV mannitol if they can't tolerate orally - asked why would they not be able to tolerate orally - said if it's CI but she kept probing for something else idk what
    b) Talked about iop drops - asked what drops and conc
    c) Pilocarpine - why not? Said need to talk to ophthal first, Talked about pressures >40, only for pupil block + plateau iris/ angle crowding — angelica asked why not, I said retrolenticular causes/iris bombe effect which could make narrow angles worse - kept probing for more, idk what.

d) They said ophthal not available for 2 hours - what's the next step? I said depends on IOP but that's the scope of my care
e) Asked what's the definitive tx - said I would refer for LPI or iridoplasty if can't do LPI straight away
f) Asked why LPI - said reverse pupillary block and allow flow of AH, prevent future attack and prophylaxis for other eye

Friday:
Examiners: Rebecca and Fiona
neuro pathway, VF, POAG, AAU, posterior uveitis

9:20
Got asked feedback and PDS
Gave a photo of endothelial dusting, got asked about this and KP and how to differentiate between the two
Why is the pigment on the endothelial layer
How does the aqueous flow in the anterior chamber
What to do if the IOP is 35
Management of PDS
Contraindications of Beta blockers